



MeticleCare

NHS & Care Provider Sales Package — Everything
you need to make a decision

Document type: Sales package — for care providers and NHS trusts

Prepared for: Prospective care provider partners

Applicant: MeticleCare Ltd

Date: August 2026

Version: 1.0

Classification: Commercial in confidence

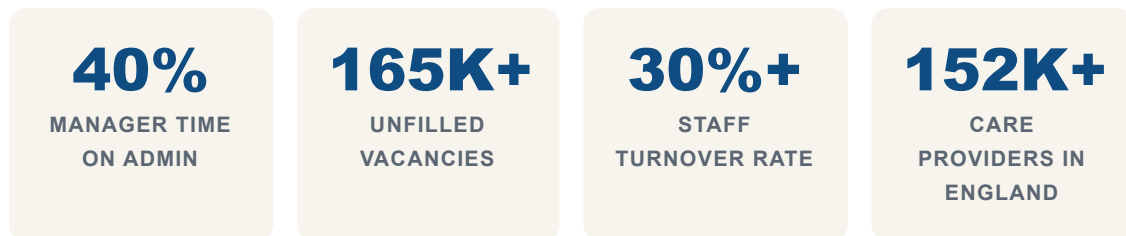
SECTION 1

Company & Product Overview

One page — problem, solution, outcomes

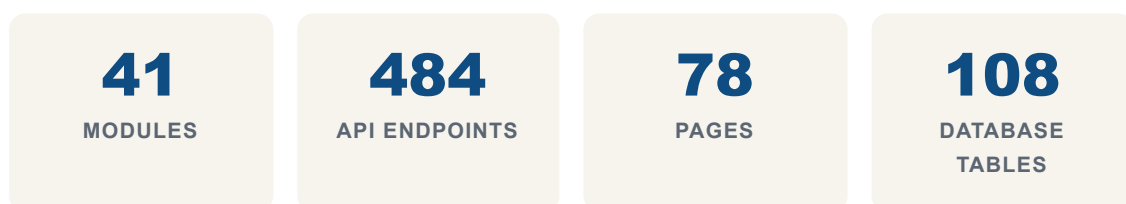
The Problem

UK care providers manage complex operations across multiple locations with fragmented tools: paper MAR charts, spreadsheet rotas, manual compliance checklists, separate training records, and ad-hoc incident forms. This creates duplicated work, missed compliance deadlines, and staff spending more time on administration than care delivery.



The Solution

MeticleCare is a single connected platform that runs the entire care working day — rota, medication, notes, compliance, leave, incidents, training — from one system. Built for UK CQC-regulated supported-living providers managing multiple locations with shift-based staffing.



Key Capabilities

Operations

- AI-powered rota planner with min safe staffing
- Full eMAR: chart grid, PRN, stock, daily counts
- Voice-to-text daily care notes

Compliance

- Automated evidence packs (KLOE)
- CQC readiness scoring from live data
- Training matrix with auto-assign
- Competency assessments

- Real-time team chat with presence
 - Mobile PWA with GPS check-in
- DSPT (Data Security & Protection Toolkit)

Outcomes

METRIC	BEFORE METICLECARE	AFTER METICLECARE
Admin time (manager)	40% of day	Under 20%
Compliance evidence gathering	Manual, weeks before inspection	Continuous, real-time
Rota management	Spreadsheets, phone calls	AI-powered, drag-and-drop
Medication records	Paper MAR charts	Digital eMAR with audit trail
Incident reporting	Paper forms, delayed	Digital with AI triage
Family communication	Phone calls, letters	Family portal with care notes

SECTION 2

NHS & Care Value Proposition

Why a provider should switch to MeticleCare

2.1 The Case for Change

The UK care sector is at a turning point. CQC's Single Assessment Framework requires continuous evidence from operational records, not point-in-time inspections. The People at the Heart of Care white paper mandates digital care records. Integrated Care Systems demand data sharing between NHS and social care. Providers who remain on paper or fragmented systems face increasing regulatory risk, rising agency costs, and staff attrition.

2.2 What MeticleCare Delivers

For Care Managers

- **One system, not five:** Replace spreadsheets, paper MARs, compliance checklists, training matrices, and incident forms with one connected platform
- **AI-powered insights:** Daily briefing tells you what needs attention — staffing gaps, overdue care plans, compliance risks, medication concerns
- **Inspection-ready evidence:** Compliance evidence packs generated automatically from real operational records
- **Real-time visibility:** See what's happening across all locations from one dashboard

For Care Workers

- **Less paperwork:** Voice-to-text daily notes, digital MAR, mobile check-in
- **Better communication:** Real-time team chat, shift notifications, rota visibility
- **Easier leave management:** Request, view, and manage leave from your phone
- **Professional development:** Training matrix, competency assessments, career progression

For Organisation Owners

- **Cost reduction:** Less admin time, less agency dependency, fewer compliance failures
- **Risk mitigation:** Continuous monitoring, automated alerts, audit trails

- **Growth enablement:** Multi-location management, scalable platform, API integrations
- **Revenue protection:** Maintain CQC ratings, reduce enforcement risk

2.3 Specific Value Drivers

VALUE DRIVER	IMPACT	ESTIMATED SAVING
Reduced admin time	Manager saves 7+ hours/week	£7,000/year per provider
Lower agency spend	Better internal utilisation	£5,000–£15,000/year per provider
Compliance automation	Continuous evidence generation	£2,000–£10,000/year per provider
Reduced staff turnover	Less admin burden, better tools	£5,000–£10,000 per retained worker
Faster discharge pathways	Family portal, care plan tracking	Reduced NHS bed occupancy

2.4 Why Now

- **CQC Single Assessment Framework:** Requires continuous evidence from operational records
- **People at the Heart of Care:** Mandates digital care records for all providers
- **Integrated Care Systems:** NHS and social care data sharing requires digital infrastructure
- **Agency cost crisis:** Rising rates make internal staffing optimisation critical
- **Workforce shortage:** 165,000+ unfilled vacancies require better retention tools

The bottom line: MeticleCare doesn't just digitise existing processes — it transforms how care organisations operate. Every compliance claim, every readiness score, every insight is backed by real records the team already keeps.

SECTION 3

Evidence Plan

What you can demonstrate now vs. what needs validation

3.1 Currently Demonstrable (Live in Production)

CAPABILITY	STATUS	EVIDENCE
Full platform (41 modules)	Live	Production deployment at meticlecare.com
AI-powered rota planning	Live	AI generation, analysis, min safe staffing enforcement
eMAR (medication records)	Live	Chart grid, PRN tracking, stock, daily counts, audit
Compliance evidence packs	Live	KLOE domain mapping, CQC readiness scoring
AI daily notes (voice-to-text)	Live	Voice input, mood analysis, safeguarding flags
Real-time team chat	Live	Channels, messages, presence, read receipts
Mobile PWA	Live	GPS check-in, voice notes, offline support
Family portal	Live	Care notes, goals, observations (consent-gated)
Incident management	Live	AI triage, CQC reportability, action tracking
Training matrix	Live	Auto-assign, expiry alerts, bulk operations
Multi-location management	Live	Location-based rota, compliance, reporting
Stripe billing	Live	Subscriptions, invoices, webhooks, payment recovery
AI compliance gap analysis	Live	CQC readiness scoring from real data
AI incident triage	Live	Severity classification, CQC notification flagging

3.2 Requires Validation (Pilot-Dependent)

CAPABILITY	WHAT NEEDS VALIDATING	VALIDATION METHOD
Clinical safety of AI outputs	AI daily notes accuracy in real care settings	Controlled pilot with 2–3 providers
Staff adoption rates	How quickly staff transition from paper to digital	Usage metrics during pilot
Compliance evidence quality	Whether CQC inspectors accept AI-generated evidence packs	Feedback from CQC inspectors
ROI validation	Actual time savings vs. projected	Before/after time studies during pilot
Integration with NHS systems	Data sharing with NHS trusts for discharge pathways	NHS integration pilot
Scale performance	Platform behaviour with 50+ concurrent users	Load testing + pilot monitoring
Multi-tenancy under load	Data isolation with multiple organisations	Security audit + penetration testing

3.3 Independent Validation Planned

- **Care Quality Commission (CQC):** Engagement with CQC inspectors to validate evidence pack quality
- **NHS Digital:** Integration testing for NHS Spine and GP system connectivity
- **Cyber Essentials:** Certification for information security baseline
- **ISO 27001:** Preparation for information security management certification
- **User research:** Structured feedback from care managers, care workers, and organisation owners during pilot

Key principle: We do not claim validated outcomes without evidence. Everything in this document is either demonstrably live in production or clearly marked as requiring pilot validation. We will not overstate capabilities.

SECTION 4

Pricing

Simple, defensible provider pricing

4.1 Pricing Model

MeticleCare uses a transparent per-organisation monthly subscription. No per-user fees. No hidden costs. No long-term contracts required (month-to-month billing available).

Starter

£99 /month

Ideal for small providers (1–3 locations) getting started with digital care management.

- ✓ Core modules: scheduling, daily notes, basic compliance
- ✓ Leave management, incidents, training
- ✓ Team chat, notifications
- ✓ Dashboard with KPIs
- ✓ Email support
- ✓ Up to 25 staff members

Professional

£299 /month

For growing providers (4–15 locations) needing full compliance and AI capabilities.

- ✓ Everything in Starter

- ✓ Full eMAR: chart grid, PRN, stock, daily counts, audit
- ✓ AI daily notes (voice-to-text)
- ✓ CQC readiness scoring and evidence packs
- ✓ Family portal
- ✓ Agency management with rate comparison
- ✓ Advanced reporting and insights
- ✓ Priority support
- ✓ Unlimited staff members

Enterprise

£499–£999 /month

For large providers (15+ locations) needing API access, custom integrations, and dedicated support.

- ✓ Everything in Professional
- ✓ API access for custom integrations
- ✓ Dedicated account manager
- ✓ SLA with guaranteed response times
- ✓ Multi-region support
- ✓ Custom AI prompts and configurations
- ✓ White-label options
- ✓ On-site training

4.2 Pricing Comparison

PLATFORM	PRICING MODEL	TYPICAL COST (20-BED PROVIDER)
----------	---------------	--------------------------------

MeticleCare Professional	Fixed monthly fee	£299/month
Person Centred Software	Per carer/day	£480–£1,280/month (20 carers × £3–£8/day)
Access Group	Per module + per user	£400–£800/month
Paper/spreadsheets	£0 + staff time	£1,167/month (40% admin time × £35K salary)

Value proposition: MeticleCare Professional at £299/month replaces £400–£1,280/month in competitor costs plus £1,167/month in admin time — a total saving of £500–£2,148/month per provider.

4.3 What's Included

INCLUDED	NOT INCLUDED (ADD-ONS)
All platform features for your tier	Custom development (quoted per project)
Unlimited locations (within tier limits)	On-site training (£500/day)
Email and chat support	Phone support (Enterprise only)
Automatic updates and new features	Data migration from legacy systems (quoted)
SSL encryption and daily backups	Custom integrations (quoted)
99.9% uptime SLA (Professional+)	

SECTION 5

Pilot Proposition

An easy way for a care organisation to trial MeticleCare

5.1 Why a Pilot

We recognise that switching care management platforms is a significant decision. A structured pilot lets you evaluate MeticleCare with real staff, real data, and real workflows — without committing to a full deployment.

5.2 Pilot Structure



Week 1: Setup & Configuration

Configure MeticleCare for your organisation: locations, departments, teams, staff roles, compliance settings. Import existing staff and service user data.

Weeks 2–4: Onboarding & Training

Onboard pilot group (recommended: 5–10 staff across roles). Training sessions for managers and care workers. Set up eMAR, rota, and compliance modules.

Weeks 5–12: Active Pilot

Use MeticleCare alongside existing systems (dual-running). Daily usage monitored. Weekly check-ins with our team. Feedback collected at Week 8 and Week 12.

Week 13: Pilot Review & Decision

Review usage data, feedback, and outcomes. Present findings to leadership. Decide: full deployment, extended pilot, or decline.

5.3 Pilot Terms

TERM	DETAIL
Duration	12 weeks

Cost	Free for the pilot period
Staff limit	Up to 15 staff members
Locations	Up to 2 locations
Features	Full Professional tier access
Support	Dedicated onboarding specialist + weekly check-ins
Data	Your data remains yours — exported if you decline
Commitment	None — walk away at any time

5.4 What We Measure During the Pilot

METRIC	HOW WE MEASURE	TARGET
Staff adoption rate	Daily active users / total pilot users	80%+ by Week 8
Time to complete daily notes	Self-reported + system timestamps	50% reduction vs. paper
Rota creation time	Manager self-reported	70% reduction vs. spreadsheet
Compliance evidence coverage	System-generated evidence packs	90%+ KLOE coverage
Staff satisfaction	End-of-pilot survey (NPS)	NPS 30+
Support tickets	Count and resolution time	Under 4 hours average

5.5 Pilot Deliverables

- Usage analytics report (Week 8 and Week 12)
- Staff satisfaction survey results
- Time-saving analysis (before/after comparison)
- Compliance evidence quality assessment
- Recommendation report with full deployment plan
- Data export if you decline (within 30 days)

No risk: The pilot is free, the data is yours, and there's no commitment. If MeticleCare doesn't work for you, we'll help you export your data and part ways professionally.

SECTION 6

Procurement Readiness

Security, GDPR, DPIA, data hosting, backups, support

6.1 Data Protection & GDPR

REQUIREMENT	STATUS	DETAIL
UK GDPR compliance	Implemented	Data processing agreements, privacy notices, consent management
Data Protection Impact Assessment (DPIA)	In progress	Full DPIA planned before pilot deployments
Data minimisation	Implemented	Collect only what's needed for care operations
Right to erasure	Implemented	Data export and deletion on request
Data portability	Implemented	CSV/JSON export of all organisational data
Breach notification	Implemented	72-hour notification process to ICO and affected parties
Record of processing	Implemented	Documented data flows and processing activities

6.2 Security

LAYER	MEASURE
Transport	TLS 1.3 encryption for all data in transit
Storage	AES-256-GCM encryption for sensitive data at rest
Authentication	JWT access + refresh tokens, MFA (TOTP), RBAC
Authorisation	Per-request permission checks, tenant isolation
API security	Rate limiting, input validation (Zod), CORS, CSP
File uploads	MIME allowlist, extension blocking, magic-byte validation

Audit logging	Centralised audit trail across 40+ mutation points
Vulnerability management	Regular dependency updates, automated scanning

6.3 Infrastructure & Hosting

COMPONENT	DETAIL
Hosting region	UK (AWS London / Azure UK South)
Database	PostgreSQL 15 with daily automated backups
Backup retention	30 days rolling, with point-in-time recovery
Uptime SLA	99.9% (Professional tier and above)
Disaster recovery	Documented RTO: 4 hours, RPO: 1 hour
SSL certificates	Automated renewal via Let's Encrypt
CDN	Static asset caching for performance

6.4 Support

TIER	SUPPORT LEVEL	RESPONSE TIME
Starter	Email support	24 hours
Professional	Email + chat support	4 hours
Enterprise	Email + chat + phone	1 hour

6.5 Certifications (Planned)

- **Cyber Essentials:** Target Q4 2026
- **ISO 27001:** Target Q2 2027
- **SOC 2 Type II:** Target Q4 2027

6.6 Insurance

- Professional indemnity insurance
- Cyber liability insurance
- Public liability insurance

Procurement note: We are a UK-registered company with UK-hosted infrastructure. All data processing occurs within the UK. No data leaves UK jurisdiction. We are happy to complete any NHS trust or local authority procurement documentation.

SECTION 7

Competitor Positioning

Why MeticleCare rather than established platforms

7.1 Competitive Landscape

COMPETITOR	FOCUS	PRICING	STRENGTHS	WEAKNESSES
Person Centred Software	eMAR + digital care notes	£3– £8/carer/day	Market leader in eMAR; NHS partnerships	Not a full platform; high per-user cost; no AI; no rota
Access Group	Enterprise care software	£200– £500+/month	Large customer base; broad feature set	Legacy architecture; slow innovation; complex pricing; poor UX
Person Centred Software	eMAR + care plans	Per carer/day	Strong in medication	Not connected; no scheduling; no compliance evidence
Paper/spreadsheets	Manual	£0 + staff time	Familiar; no learning curve	No audit trail; compliance risk; high admin burden

7.2 MeticleCare Differentiation

DIFFERENTIATOR	METICLECARE	COMPETITORS
Full platform (not point solution)	41 modules, one system	Typically 3–5 modules
AI-first architecture	5 AI agents, event-driven intelligence	No AI or bolt-on features
Predictable pricing	Fixed monthly fee, no per-user charges	Per carer/day or per module

Modern architecture	TypeScript, React, PostgreSQL	Legacy tech stacks
UK-built and hosted	UK company, UK data centres	Often US/EU hosted
Mobile-first PWA	Offline-capable, GPS check-in	Desktop-first or basic mobile
Real-time collaboration	Socket.IO chat, live presence	Email-based communication
Family portal	Consent-gated access to care records	Rarely included
CQC evidence automation	Continuous evidence from operational records	Manual evidence gathering
Open API	484 endpoints, full API access	Limited or no API

7.3 Objection Handling

OBJECTION	RESPONSE
"You're a startup — what if you fail?"	Your data is yours. We provide full data export at any time. We also maintain daily backups with 30-day retention. If we cease trading, you retain your data.
"We already use Person Centred Software"	MeticleCare replaces the need for multiple point solutions. One platform instead of 3–5. Lower total cost. Plus AI capabilities that PCS doesn't offer.
"We're not ready for AI"	AI is optional. You can use MeticleCare without any AI features. When you're ready, AI capabilities are there — but they're never required.
"Our staff won't adopt new software"	Our pilot includes dedicated onboarding and training. We've designed the UX for care workers on phones and tablets — not desktops. Voice-to-text means less typing, not more.
"What about CQC?"	MeticleCare generates CQC evidence automatically from operational records. The Single Assessment Framework requires continuous evidence — we provide it.
"We need NHS integration"	NHS Spine integration is on our roadmap for Q2 2027. We're building toward it with pilot partnerships.

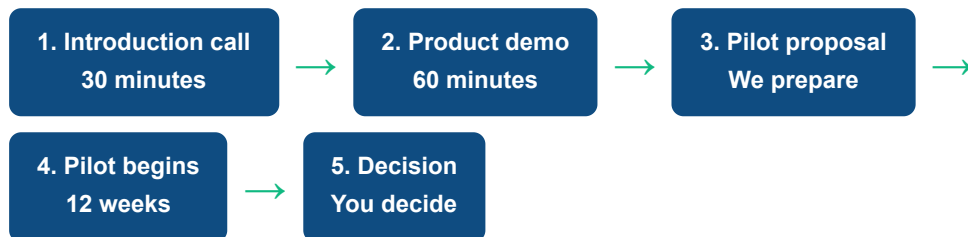
Positioning statement: MeticleCare is the only UK care management platform that combines a full operational system (41 modules) with AI-first intelligence, predictable pricing, and modern architecture. Competitors offer point solutions at higher cost. We offer a complete operating system at a predictable price.

SECTION 8

Next Steps

How to get started

8.1 Immediate Actions



8.2 What We Need From You

- 30-minute introduction call with the care manager or operations director
- Overview of your current systems and pain points
- Number of locations, staff, and service users
- Compliance requirements (CQC, CIW, Care Inspectorate, RQIA)
- Decision-maker identification

8.3 What We'll Deliver

- Personalised demo based on your specific workflows
- Tailored pilot proposal within 5 working days
- Data migration plan (from existing systems)
- Training schedule for pilot staff
- Dedicated onboarding specialist for the pilot period

Contact: Opeyemi Olorunfemi, Founder & CEO — MeticleCare Ltd

Email: opeyemi@meticlecare.com

Web: meticlecare.com